

Medical Domain: Clinical Actionability

The representative failure is not poor wording, but an unsafe and under-specified clinical handoff.

startUP-V1-027

1 Domain pressure

Why medical is representative

Medical tasks expose whether an agent can turn domain knowledge into safe, executable decisions.

Tasks

21

medical

Runs

42

model outputs

Avg.

61.89

domain score

Top recurring medical errors

- 151 professional rule misread
- 141 format / object constraint miss
- 100 unusable output quality

Representative domain risk

Small inversions in medication continuation, restart timing, or discharge step-down can change clinical safety. This makes surface fluency a weak proxy for success.

2 Medical failure chain

1 Conflicting clinical priorities

Upper GI bleeding conflicts with recent DES thrombosis protection.

2 Core antiplatelet rule is inverted

Clopidogrel should be protected, but the output stops it before hemostasis.

3 Timing criteria drift

Apixaban is restarted at day 5-7 instead of no earlier than day 7.

4 Consensus is not made explicit

No clear unified conclusion that cath lab is not the immediate priority.

5 Discharge plan remains risky

The plan defaults toward high-bleeding-risk triple therapy instead of step-down.

Domain-specific issue: fluent MDT prose masks unsafe rules.

3 Representative case evidence

What looked complete

A five-part MDT plan with stages, roles, escalation, and discharge

What failed

Key clinical safety constraints were missed or inverted.

Artifact-level judge findings

P2Y12

Needed: clopidogrel uninterrupted**Model:** stopped with aspirin before hemostasis

Cath lab

Needed: state no immediate cath lab**Model:** not restated as unified decision

DOAC

Needed: restart day 7-10 only**Model:** restart day 5-7

Discharge

Needed: step down from triple therapy**Model:** defaults toward triple therapy

Why the case is domain-specific

The errors are clinically consequential even though the answer is organized and

Failed checks: 21 / 34**Score: 27.68 / 100**

Why this supports the StartupBench story

Domain expertise

The judge distinguishes clinical safety from fluent medical-sounding prose.

Workflow realism

The task asks for a hospital-usable unified MDT plan, not a generic answer.

Acceptance examples

Specific medication timing and handoff constraints make errors observable.

Domain takeaway

Medical failures concentrate in clinical actionability: agents can assemble a convincing plan while missing the exact safety constraints that make it usable.